“What Should We Tell Them About Her?” Eight Questions Families Ask When New Caregivers Take Over

General answers about sharing her routines and preferences, and who to ask when the question is about her.

A new team is about to look after your wife, and you're wondering how much about her they need to hear, and who should hear it. Families in this spot ask much the same questions. Here are eight, answered in general terms.

None of the answers is about your wife in particular. Where a question needs someone who knows her, the answer says who to ask. Some of the sources were written for nursing homes or for families living with dementia; we've used them only for what they say about introducing a person to new caregivers.

1 What should new caregivers know about her?

Start with what someone could use on the first day: the name she likes to be called, how her morning and evening go, what she likes to eat and drink, the people and activities that matter to her, how she likes to be spoken to, and what helps when she's unsettled. The Alzheimer's Association's guidance on working with care providers lists likes, dislikes and morning and evening routines among the details to share, and says caregivers should treat a whole person, not a patient. Before the page goes anywhere, ask her what she's comfortable having on it. [1]

> **ASK A PROFESSIONAL WHEN** you can't tell whether a detail is a preference or something medical. Ask her doctor or nurse which it is before it goes on the page.

2 Which routines help her feel comfortable?

She is the one who knows, so start by asking. A workbook from The Conversation Project uses the question “What does a good day look like for you?” and suggests taking it in small pieces: it's always okay to skip a question, and to take a break if she gets tired. Then add what you've seen yourself, the cup she reaches for or the radio on before breakfast, and mark those lines as yours. [2]

> **ASK A PROFESSIONAL WHEN** she isn't able to tell you, or what she says and what you see don't match. Write down what you have, say whose account it is, and bring it to the people assessing her.

3 How do we communicate her preferences clearly?

On one short page, in short lines, with her words where you have them. Keep her preferences in one place and anything a clinician has instructed in another. Put a name and a date at the bottom. The Alzheimer's Association's Personal Facts and Insights form ends with “Completed by” and “Date completed” lines, and The Conversation Project's workbook notes that preferences often change as health changes or as time goes by, so you'll want to be able to tell this version from the next one. [3] [2]

> **ASK A PROFESSIONAL WHEN** you don't know what form the new team wants it in. Ask the person handling her admission or the start of her care.

4 Who on the new team should get the page?

Ask them. The answer depends on the setting. If she's moving into a nursing home, the National Consumer Voice lists the staff worth knowing, among them the certified nursing assistants, the nurses, the director of nursing and the director of social services, and notes that the nursing assistants do the direct hands-on work with residents. Wherever she'll be, ask who will read the page, where it will be kept, and whether the people with her day to day will see it. [4]

> **ASK A PROFESSIONAL WHEN** you've handed it over and can't tell whether it reached the people who are with her each day. Ask the nurse in charge or the supervisor.

5 Can't we just tell each caregiver ourselves?

You can, and you'd be telling it again to every new person. A page says the same thing to everyone, including the people you never meet. Keep talking with the staff too. The Family Caregiver Alliance, writing about residential care, suggests building a relationship with the staff who care for her and greeting them by name, and reminds families that they are still part of the care team. [5]

> **ASK A PROFESSIONAL WHEN** something on the page keeps being missed. Raise it with the staff involved, or ask who else should have a copy.

6 What if she'd answer differently than we would?

Then her answer is the one that goes on the page. The National Consumer Voice tells families of nursing home residents that the resident's needs and wishes come first, and that what they communicate "may be different than what you think is best for your loved one." AARP's family caregiving guide says every caregiving plan must center on the wishes of the person receiving the care. If you see something differently, note your view separately and mark it as yours. [4] [6]

> **ASK A PROFESSIONAL WHEN** she can't say what she prefers, or the difference touches her health or safety. That isn't for the family to settle alone.

7 How do we keep what she likes separate from what her clinicians have told us?

With two lists: what she likes and dislikes goes on one, in her words. What a doctor, nurse or dietitian has told you goes on the other, with who said it and when. An allergy belongs on the second: MedlinePlus describes a food allergy as an abnormal immune system reaction to certain foods, which makes it a medical matter and not a dislike. Don't merge the lists, and don't turn a preference into a rule or an instruction into a preference. [7]

> **ASK A PROFESSIONAL WHEN** you aren't sure whether something is an allergy, an intolerance, a prescribed diet or simply a dislike. Her doctor, nurse or dietitian can tell you which.

8 Will she be able to keep her routines in the new place?

That depends on where she's going and on her care, and no guide can promise it. For nursing homes, Medicare's guide for residents says the home must recognize and respect a resident's preferences when giving care, and that residents have the right to make their own schedule, like when they go to bed, wake up and eat meals. That guide covers nursing homes; it doesn't describe assisted living or care at home, so in those settings ask the provider how her preferences are recorded and used. In any setting, a routine on your page is a request, not a care order. [8]

> **ASK A PROFESSIONAL WHEN** one of her routines seems to be at odds with something a clinician has instructed. That is for her care team to work through with her.

What this guide doesn't do

It doesn't answer any question about your wife herself. What she can eat, how her routines fit around her care, and how to learn her preferences if she can't say them all need the people who assess and care for her.

Ready for the next step?

You have plain answers to the general questions. The care interview is where they become about her: it asks about her preferences across every part of the day, alongside her care needs and the support around her, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Alzheimer's Association, Working with Care Providers
2. The Conversation Project (Institute for Healthcare Improvement) and Ariadne Labs, What Matters to Me: A Workbook for Better Health Care
3. Alzheimer's Association, Personal Facts and Insights (form)
4. National Consumer Voice for Quality Long-Term Care, Family Involvement in Nursing Homes (fact sheet)
5. Family Caregiver Alliance, Residential Care Options: Caregiving Doesn't End When Your Loved One Moves
6. AARP, Prepare to Care: A Planning Guide for Families
7. MedlinePlus (National Library of Medicine), Food Allergy
8. Centers for Medicare & Medicaid Services, Your guide to living in a nursing home (CMS Product No. 12217)